

Overview, Characteristics, and Causes

What borderline personality disorder is, how it is diagnosed, and what the research says about where it comes from.



SECTION ONE

What borderline personality disorder is

Borderline Personality Disorder (BPD) is a complex psychiatric condition primarily characterized by pervasive instability in emotion regulation, interpersonal relationships, self-image, and impulse control. Historically, the term was coined by Adolf Stern in 1938 to describe individuals who occupied a “borderline group of neuroses” between neurotic and psychotic classifications. It was formally included in the Diagnostic and Statistical Manual of Mental Disorders (DSM-III) in 1980.

BPD is recognized as a developmental disorder whose symptoms typically begin to manifest during adolescence or early adulthood. It represents a core dysfunction of the emotion regulation system, often conceptualized as experiencing emotions with heightened intensity, heightened sensitivity, and a slow return to emotional baseline.



The shift is in intensity and recovery time, not in who someone is.

SECTION TWO

Characteristics and features

DSM-5-TR diagnostic criteria

According to the DSM-5-TR, a diagnosis of BPD requires a pervasive pattern of instability in interpersonal relationships, self-image, and affects, along with marked impulsivity, indicated by **at least five** of the following nine criteria.

- 1 Frantic efforts to avoid real or imagined abandonment.
- 2 A pattern of unstable and intense interpersonal relationships characterized by alternating between extremes of idealization and devaluation.
- 3 Identity disturbance: markedly and persistently unstable self-image or sense of self.
- 4 Impulsivity in at least two areas that are potentially self-damaging, such as spending, sex, substance abuse, reckless driving, or binge eating.

- 5 Recurrent suicidal behavior, gestures, or threats, or self-mutilating behavior.
- 6 Affective instability due to a marked reactivity of mood, such as intense episodic dysphoria, irritability, or anxiety usually lasting a few hours and rarely more than a few days.
- 7 Chronic feelings of emptiness.
- 8 Inappropriate, intense anger or difficulty controlling anger, such as frequent displays of temper, constant anger, or recurrent physical fights.
- 9 Transient, stress-related paranoid ideation or severe dissociative symptoms.

The ICD-11 borderline pattern

The International Classification of Diseases (ICD-11) describes the “borderline pattern” descriptor as a pervasive pattern of instability in relationships, self-image, affects, and marked impulsivity, incorporating features such as emotional reactivity, chronic emptiness, recurrent self-harm, and transient dissociative or psychotic-like features during states of high affective arousal.

Core behavioral and psychological patterns

- **Emotional dysregulation.** Heightened reactivity, profound emotional storms, and difficulty returning to an emotional baseline. Individuals often report feeling as though they have “no emotional skin.”
- **Interpersonal dysregulation.** Intense, unstable relationships, severe rejection sensitivity, and extreme fears of abandonment.
- **Behavioral dysregulation.** Risky impulsivity, non-suicidal self-injury, and elevated rates of suicidal gestures or attempts.
- **Cognitive and self-dysregulation.** Impaired mentalizing, meaning difficulty inferring and understanding one's own and others' mental states, alongside automatic non-reflective thinking, an unstable sense of self, and persistent emptiness.
- **Parenting and relational dynamics.** Parents with BPD traits may struggle with containing emotional responses, deploying intense control or over-planning to cope, and at times inadvertently expecting caregiving or emotional support from their children.

SECTION THREE

Where it comes from

BPD develops through a complex, transactional biosocial process involving biological vulnerabilities, neurobiological mechanisms, and environmental factors interacting over time. No single cause has been identified, and no single cause is expected.

Biological and neurobiological factors

- **Genetic endophenotypes.** While a single genetic anomaly has not been pinpointed, genetic influences are evident. Polymorphisms in the tryptophan hydroxylase (TPH) gene have been linked to impulsive aggression and suicidal behavior in BPD.
- **Neuroendocrine dysregulation.** Dysfunctions in the hypothalamic-pituitary-adrenal (HPA) axis lead to chronic hyperarousal and altered cortisol levels during stress.
- **Brain structural and functional alterations.** Neuroimaging reveals hyperactivity in the amygdala and hippocampus during emotional processing, paired with deficits in habituation to negative stimuli and reduced top-down prefrontal control.

Environmental and psychosocial factors

- **Invalidating environments.** Linehan's biosocial model emphasizes that an invalidating childhood environment, where emotional expressions are dismissed, trivialized, or punished, interacts reciprocally with biological emotional vulnerability to drive core BPD symptoms.
- **Adverse childhood experiences.** Severe early trauma, including childhood physical, emotional, or sexual abuse, neglect, and parental mental illness, is a strong predictor for BPD development.
- **Familial and peer disruption.** Dysfunctional family dynamics, such as guilt induction, psychological control, and parental marital conflict triangulation, along with severe peer victimization or bullying, significantly increase the risk of early-onset BPD traits.

A vulnerability is not a verdict.

The biosocial model describes an interaction, not a sentence. The same research that identifies these risk factors is the research that established BPD as one of the more treatable conditions in the manual. The next guide in this series covers what treatment actually looks like.



ABOUT THE GUIDE

Meet Marley

This project started with one family trying to make sense of a diagnosis.

Marley grew up inside a family where emotions ran high and nobody had a word for why. That question led her to spend a summer buried in clinical research on borderline personality disorder, and to build this series of guides for other families doing the same thing hers did.

Each guide is written from the peer-reviewed literature rather than from anecdote, and every claim is traceable to the sources listed at the back. They are free, and they are meant to be shared with the people who need them.

“Nobody in my family had the language for what was happening. These guides are the thing I wish someone had handed us.”

SOURCES

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